

Surgical and Procedural Safety

Universal Protocol, Time-Out, and Wrong-Site Prevention

Regulation: 42 CFR §482.51 | Joint Commission NPSG 01.03.01

POLICY OVERVIEW

Surgical safety policies are designed to prevent wrong-patient, wrong-site, and wrong-procedure errors. The Universal Protocol, developed by The Joint Commission, provides a standardized framework that must be followed before every invasive procedure.

KEY REQUIREMENTS

1 Pre-Procedure Verification

A pre-procedure verification process must confirm the correct patient (two identifiers), correct procedure, correct site/side, and availability of required documents, imaging, equipment, and implants before the patient leaves the pre-operative area.

2 Site Marking

The operative or procedure site must be marked by the licensed practitioner who will perform the procedure. Marking must be done with the patient awake and aware when possible. The mark must be visible after prepping and draping.

3 Time-Out Procedure

Immediately before starting any invasive procedure, the entire team must pause for a time-out. All team members must actively participate and confirm: patient identity, procedure, site/side, patient position, and availability of critical equipment.

4 Surgical Count Policy

Sponge, sharps, and instrument counts must be performed and documented according to AORN standards at the beginning of the procedure, before closure of any body cavity, at wound closure, and at shift change.

5 Specimen Management

Surgical specimens must be labeled immediately in the sterile field, with the patient's name, MRN, specimen source, and laterality. Chain of custody documentation must accompany specimens to pathology.

COMPLIANCE NOTES

All team members have equal authority to stop a procedure if safety concerns arise. A culture of psychological safety is essential. Retained surgical item events are reportable sentinel events and require root cause analysis.

COMMON SURVEY CITATIONS

Cited under §482.51(b) – Surgical services policies, and NPSG.01.03.01 – Eliminate wrong-site, wrong-procedure, wrong-person surgery.